

COOK COUNTY PEDIATRIC SPECIALTY CLINICS

Division of Pediatric Endocrinology, Diabetes & Metabolism

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OUTPATIENT CONSULTATION NOTE — PEDIATRIC ENDOCRINOLOGY

Patient: Marcus D. Whitfield **MRN:** PED-2025-04471
DOB: 03/03/2016 **Age:** 9 years, 6 months **Sex:** Male
Date of Visit: 09/10/2025 **Visit Type:** New Patient Consultation
Referring Clinician: Anita Sharma, MD — Cook County Pediatrics
Attending: Lillian Chen, MD, FAAP — Pediatric Endocrinology

REASON FOR REFERRAL

Marcus is a 9-year-old male with a complex medical and psychiatric history referred for evaluation of abnormal fasting glucose and dyslipidemia identified on routine screening in the context of chronic second-generation antipsychotic therapy (risperidone).

HISTORY OF PRESENT ILLNESS

Recent fasting glucose was 102 mg/dL. Fasting lipid panel demonstrates LDL 118, HDL 38, TG 142, TC 186. HbA1c today is 5.4%. Marcus has been on risperidone 1.5 mg nightly for approximately 18 months for irritability associated with ASD. He is also on infliximab for suspected Crohn's disease. He is significantly underweight (BMI 14.9, <5th percentile) yet demonstrates metabolic derangements typically associated with overweight or obesity. No family history of type 2 diabetes is available (biological father unknown, maternal history limited).

MEDICATIONS

- Risperidone 1.5 mg PO QHS
- Methylphenidate ER 27 mg PO QAM
- Clonidine 0.1 mg PO QHS
- Sertraline 50 mg PO QHS
- Infliximab 5 mg/kg IV q8 weeks (GI-directed)
- Budesonide/formoterol inhaler BID
- Albuterol MDI PRN
- Ferrous sulfate 325 mg daily
- PEG 3350 17 g daily
- Melatonin 3 mg QHS (OTC)

EXAMINATION

Vital Signs: Weight 25.6 kg, Height 131 cm. BMI 14.9 kg/m² (<5th %ile for age/sex).

General: Alert, cooperative male. Thin habitus. Skin: No acanthosis nigricans at nape, axillae, or groin. Multiple cafe au lait macules on trunk. No violaceous striae. Neck: Supple, no thyromegaly. Abdomen: Soft, mildly tender, no hepatomegaly.

ASSESSMENT

1. Prediabetes in the context of second-generation antipsychotic use — HbA1c 5.4% is below the prediabetes threshold (5.7%) but fasting glucose 102 suggests impaired fasting glucose. The metabolic derangement in this underweight child is best explained by three converging mechanisms: risperidone-induced receptor-mediated insulin resistance (5-HT_{2C} and H1 antagonism affect glucose homeostasis independent of weight gain), active inflammatory cytokine burden from Crohn's disease (TNF-alpha, IL-6 impair insulin signaling), and FASD-related metabolic dysregulation (documented in the FASD literature as an independent contributor to glucose and lipid abnormalities).

This presentation should not be dismissed or attributed solely to undernutrition. The metabolic derangements are real and mechanistically distinct from those driven by obesity. Conversely, interventions appropriate for overweight-associated metabolic syndrome (caloric restriction) would be harmful in this child, whose primary nutritional goal remains adequate intake for growth and development.

2. Dyslipidemia — age-inappropriate lipid profile consistent with antipsychotic effect and inflammatory contribution. Low HDL and elevated triglycerides are the pattern most consistent with antipsychotic-associated metabolic effects.

3. Underweight status — BMI <5th percentile. Growth monitoring ongoing through PCP and GI.

PLAN

- No initiation of metformin at this time. HbA1c 5.4% does not meet threshold for pharmacologic intervention. However, if HbA1c rises above 5.7% or fasting glucose exceeds 110 on repeat testing, low-dose metformin should be reconsidered.
- Lifestyle counseling: age-appropriate physical activity encouraged. Dietary guidance deferred to GI and nutrition given competing priorities (growth vs. metabolic risk).
- Repeat fasting glucose, HbA1c, and fasting lipid panel in 6 months.
- ADA/APA antipsychotic metabolic monitoring protocol should be followed by PCP. Baseline labs and monitoring intervals reviewed with referring provider.
- Follow-up in pediatric endocrinology in 6 months, or sooner if clinical status changes.

Lillian Chen, MD, FAAP

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